Global study shows varied trend in obesity worldwide and criticizes the term epidemic

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Obesity rates have increased on almost every continent over the past 40 years, but have plateaued in some regions. Trends vary among adults, children and adolescents, reaching the highest level among girls in 110 countries

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Obesity is growing rapidly in low- and middle-income countries, driven by social, economic and technological factors, with minimal effects from the use of injectable weight-loss drugs – Photo: Ernesto Rosas- Pexels

- The advance of obesity in 2024 was the highest in 45 years for developing countries
- Growth stabilized below 25% in Western Europe, but ranged up to 43% in the United Kingdom, Canada and the United States
- The total number of cases continued to grow in countries where prevalence was already high
- While Japan reduced cases among school-age children to 10%, in Brazil the adult male population showed an accelerated increase in obesity rates

An international network of scientists has just presented an analysis of obesity dynamics from 1980 to 2024 in 197 countries, using a vast dataset and revealing heterogeneous trends across different genders, age groups and countries. The unprecedented study, [published on May 13 in the journal *Nature*](https://www.google.com/search?q=https://www.nature.com/articles/s41586-026-10383-0), measured the annual variation in the percentage of the population with obesity and highlighted that not only income, but also social, economic and technological factors influence variations in obesity worldwide, even in countries in the same region.

“For more than three decades we have framed this as a global epidemic. And we wanted to see whether there is any other layer of complexity beneath this global, single and uniform framing”, said Majid Ezzati, professor at Imperial College London and corresponding author of the study, in a press briefing. “If we look in detail at individual countries, instead of observing long periods of four or five decades, we can see that there has been a major change in the way obesity is behaving, which does not exactly fit the picture of a global epidemic.”

The research gathered data from 4,050 population-based studies, covering 232 million participants aged five years or older, including weight and height measurements. The work took into account age patterns, differences among populations living in urban and rural areas, as well as possible variations in data sources, which were normalized using statistical techniques. Obesity was defined by a Body Mass Index (BMI) of 30 kg/m² in adults. In children and young people, the criterion was having weight two levels above, 2 standard deviations, the growth average defined by the World Health Organization (WHO).

“Epidemic implies affecting everyone, regardless of individual control, as occurred with covid-19. The increase in obesity is a worldwide phenomenon, with significant regional differences”, explained physician Paulo Andrade Lotufo, professor at the Medical School (FMUSP) at USP and one of the authors of the work. Speaking to **Jornal da USP**, he stated that the work identified a variation marked by social factors.

> *There is a slowdown in the increase in obesity in wealthier countries and an acceleration in poorer countries. The major marker is social everywhere in the world. — Paulo Lotufo*

“Calling this phenomenon globally an ‘epidemic’ ends up masking these important differences and oversimplifying a complex and dynamic process”, argued Alicia Matijasevich, professor at FMUSP and also a co-author of the article published in *Nature*. She is one of the researchers of the 2004 Pelotas Birth Cohort, a world-reference epidemiological study that was used as a database for the current work.

The research entitled *Obesity rise plateaus in developed nations and accelerates in developing nations* was conducted through the NCD Risk Factor Collaboration (NCD-RisC), a network of nearly 2,000 scientists who investigate the main risk factors for non-communicable diseases. According to the group, GLP-1-based medications, such as Wegovy and Ozempic, had a minimal effect on current trends due to historically low coverage, but are seen as an influential factor for the future.

Slowdown and pateau

The picture of obesity drawn by the study shows significant variation among countries, age groups and sex. Prevalence indicates the general scenario of obesity. It measures the proportion of the population that has the condition during a specific period. In addition to quantifying how obesity changed over time, the NCD-RisC study presented an analysis of the speed of these changes, which is defined as the absolute annual change in prevalence.

“It is like driving a car: how far you have gone, but what is your current speed? So, how much has obesity increased, but how much is it increasing now? And again, the speed has been zero or, in some cases, perhaps even negative for a few decades”, pointed out Majid Ezzati.

Overall, high-income countries—such as those in Western Europe, North America and Australasia—showed an increase in obesity in the early 1980s, but most managed to stabilize with varying prevalence. In Western Europe, for example, obesity prevalence stabilized between 11% and 23% for adults and between 4% and 15% for children and adolescents.

Other high-income countries, such as Japan, showed a less pronounced increase and stabilized at lower prevalence levels, especially among women. The first slowdown occurred around 1990 in Denmark, for both sexes, followed by other countries such as Iceland, Switzerland, Belgium and Germany. France was one of the only countries where obesity prevalence among boys did not increase between 1980 and 2024.

However, some high-income countries, such as Finland, Australia and Sweden, continued to see steady or accelerated increases in obesity. Even in countries where obesity cases stopped increasing, such as the United States, obesity stabilized at endemic prevalence levels considered concerning: between 19% and 25%.

According to the research, these differences are often explained by common generalizations, such as the availability and marketing of ultra-processed foods, low levels of energy expenditure and physical activity—factors included under “macrotrends,” such as urbanization. Alone, however, they do not fully support the variable trends.

Other factors, such as cultural norms, socioeconomic status and access to healthcare, probably play more significant roles. “These may influence what and how much children, adolescents and adults eat at home and in social environments, how much they participate in sports, play and active transportation, social norms and perceptions related to body image, and the mismatch between ideal, perceived and actual body weight”, the researchers stated in the work.

“In the case of women, we do not know whether it may be related to physiology, but inevitably a large part of this is related to gender roles”, said Majid Ezzati. The Imperial College London professor highlighted a Spanish study that attributed the recent decline in obesity cases among women to changes in social roles. “With more autonomy, greater participation in the workforce, women’s behavior regarding diet and physical activity may have changed”, he commented.

Despite the stabilization trend, the study points to some countries that managed to slow the increase in obesity among younger people, including Croatia and Slovenia, for both sexes, and the Czech Republic and Montenegro, for boys. In middle-income countries such as Kyrgyzstan and Kazakhstan, school-age children and adolescents, especially girls, did not experience the increase in obesity observed elsewhere over these four decades; on the contrary, there was a decline in the mid-2000s.

Graphs show phenotypes of national obesity trajectories in children and adolescents, left, with distribution for girls (a) and boys (b) from 1980 to 2024; and in adults, right, with distribution for women (a) and men (b), in the same period – Image: Extracted from the article

The Global South moving in the opposite direction

By contrast, most low- and middle-income countries experienced continuous growth in obesity prevalence, often surpassing high-income countries. This acceleration was identified in regions such as Asia, Africa, Latin America, Caribbean countries and Pacific island nations.

In these places, the speed of obesity was higher in 2024 than in any other year since 1980 for girls in 110 countries and boys in 91 countries. Some of the countries with the highest speeds were Tonga and Samoa, in Oceania, and Peru, in South America.

Among adults, the increase in obesity was also highest in 2024 for women in 84 countries and for men in 109 countries, mainly in low- and middle-income regions.

“In some poor countries, obesity still has relatively low prevalence, but it is growing rapidly; in others, obesity has already reached very high prevalence levels and continues to rise. This is related to economic and social transformations that have occurred in these countries, such as: mechanization of work and transportation; increased purchasing power; expansion of the commercialization and marketing of ultra-processed foods; changes in dietary and physical activity patterns”, observed Alicia Matijasevich. The FMUSP professor explained that one of the arguments of the work is the lack of public policies that ensure regular access to healthy and minimally processed foods.

> *Many developing countries focused their efforts on combating malnutrition, and there was little regulatory response to the advance of marketing of unhealthy products, such as sugary drinks. – Alicia Matijasevich*

In Brazil, the absolute annual change in obesity prevalence showed an accelerated pattern over the 45 years analyzed by the research. The country was among the Latin American countries analyzed in which, for adult men, obesity prevalence increased at a rate of more than 0.5 percentage points in 2024. For comparison purposes, in the same year, most high-income countries, such as Italy and Portugal, had speeds that were sometimes indistinguishable from zero.

Asked about the obesity situation in Brazil, Majid Ezzati told **Jornal da USP** that the study observed a more significant increase in obesity among men, a case particularly different from what occurs in the other countries observed. “Some of the lowest levels are among women and some of the highest absolute levels in the world are also among women. Men vary among countries, both in terms of how high obesity becomes and how quickly it is changing”. For him, the gender issue “is absolutely fascinating and needs to be investigated further.”

A double-edged sword

Middle Eastern countries also showed differences in obesity trends. In Kuwait, for example, it has stabilized, but continues to rise in Iran, Oman and Saudi Arabia.

In East Africa, including countries such as Ethiopia and Rwanda, prevalence among adults in 2024 was below 5%, but reached 30% to 40% in some countries in Central Europe, such as Romania and the Czech Republic, and in Latin America, such as Brazil.

For the authors of the article, the varied dynamics of obesity worldwide require personalized policy interventions adapted to the realities of each country, rather than generalized approaches. The slowdowns and reversals observed in some high-income countries demonstrate that the increase in obesity can not only be contained, but stabilized at lower prevalence levels.

“The increase in economic growth has recently led to higher household income, and purchasing power to access unhealthy foods is becoming much easier”, said Guha Pradeepa Rajendra Prabhu, from the Madras Diabetes Research Foundation in India and also a co-author of the article. The researcher highlighted the rapid transition from the intake of traditional foods, with diets rich in whole grains, vegetables and legumes, to processed foods high in added sugars, salt and saturated fat in underdeveloped countries.

> *Replacing about 5% to 10% of carbohydrates with 25% protein would decrease the risk of developing metabolic disorders, both in urban and rural areas.* *– Guha Pradeepa*

Guha Pradeepa also recalled that screen use, by adults and children, has deepened sedentary lifestyles in poorer countries. “Due to rapid urbanization, there is little space for walking, cycling or even playing. Children do not play and there is greater dependence on motor vehicles.”

“These structural inequalities play a central role in obesity trends”, concluded Alicia Matijasevich.

The article *Obesity rise plateaus in developed nations and accelerates in developing nations* is now available on the [*Nature* website](https://www.google.com/search?q=https://doi.org/10.1038/s41586-026-10383-0).

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